

CLASSIC VISIONS . BARBADOS

Dispenser Handbook

Lens Portfolio & Dispensing Reference

If you read this cover to cover, by the end you will know our products, our current names for them, how to fit and dispense them, and how to solve many of the problems that walk through the door.

Edition 2026.1 · For optical professionals and dispensing staff

START HERE

Welcome — how to use this manual

This is not a price list and it is not marketing. It is the reference you can sit down and read to become fluent in what Classic Visions sells and how to put it on a patient's face correctly. Keep it near the counter.

- Products first: learn the families and, above all, the product names. A patient or an order with the wrong product or option name causes remakes.
- Then the craft: understanding, fitting, dispensing and troubleshooting progressive lenses — the lenses that generate the most questions and the most returns.
- Then the reference: single vision, bifocals, materials, coatings, tints, Chemistrie, Crizal and Eyezen and a quick-recommendation cheat sheet.

This is a draft document and will improve over time.

ONE RULE ABOVE ALL

We have moved on from older, mixed naming. Always use the CURRENT house names in this manual. Do not revert to supplier design names or retired labels — the next section is the single source of truth.

The naming system — how to say it

Every Classic Visions lens family sits on one four-step ladder: Best, Better, Good, Adept. To order you need only the TIER and the MATERIAL — the design name follows. Learn this table.

FAMILY	BEST	BETTER / GOOD	ADEPT (ENTRY)
Progressive	Endless Steady	Essential Steady / Classic PAL	Conventional PAL
Single Vision	Endless SV	Eyezen (enhanced)	Conventional SV
Bifocal / Trifocal	Endless Bifocal	Wide-Seg Blended Bifocal	Conventional BF / TF
Occupational	Endless Office	—	—
Active / outdoor	Endless Sport	—	—
Digital comfort	Endless Anti-Fatigue	—	—
Photochromic	ZenVue Darkun / Transitions	Xtractive / GEN S	—
Anti-reflective	Super AR+ / Crizal Sapphire	Blue Defense AR+	Standard AR

Say this, not that

DON'T SAY	SAY	WHY
"Adapt" / "Basic" progressive	Conventional PAL (Adept tier)	Locks the tier language patients and orders share
"Classic" on its own	Classic PAL	"Classic" alone is ambiguous with the brand name
A supplier's internal design name	The house Endless name	Customers buy Classic Visions names, not supplier codes
"No-line bifocal"	Progressive (PAL) / Endless Bifocal	Avoids mixing categories; sets the right expectation
"Transitions" for any photochromic	The specific product (GEN S, Xtractive, Darkun)	They behave differently — see the photochromic section

COACHING NOTE FROM THE TROUBLESHOOTING GUIDE

"No one has ever explained that before" is a real problem in optics. Using consistent, correct names — and explaining them — is the fastest way to look expert and prevent complaints.

Definition of Adept:

Adept: adjective, very good at doing something.

Progressive lenses — the range

Personalized, digital free-form progressives built to each patient's needs, plus a dependable conventional option. Choose the tier by patient and budget.

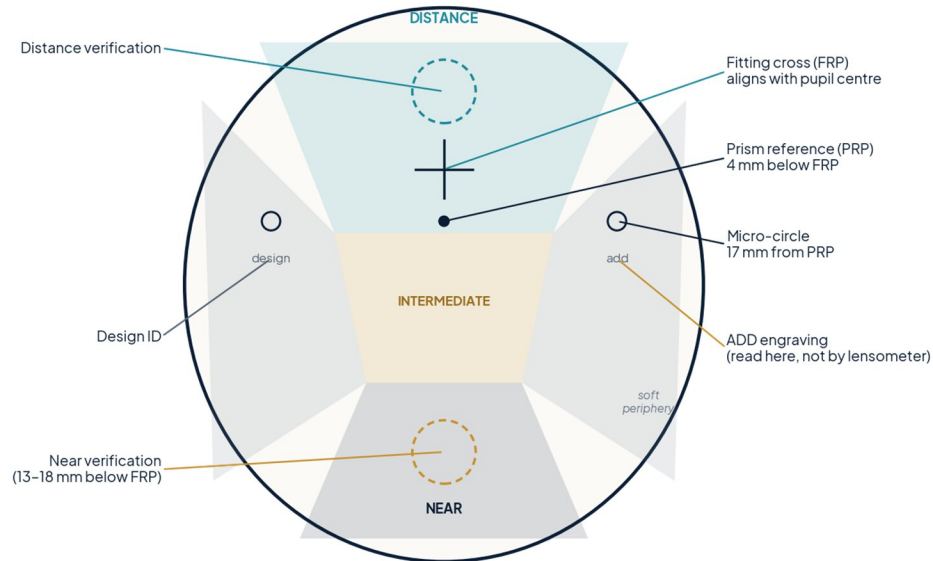
BEST Endless Steady	Fully personalized free-form. Considers the wearer's accommodation for exceptional clarity, near-zero peripheral blur and superb screen vision. MFH 14–18 mm · IOT Digital Ray-Path 2 + Steady · Personalized
BETTER Essential Steady	Non-compensated free-form with a wide field of view and higher image stability. Outstanding value in its class. MFH 14–18 mm · Steady Methodology
GOOD Classic PAL	Entry-level digital free-form. Balanced near and distance fields, clearly superior to a conventional lens. MFH 14–18 mm · Digital surfacing
ADEPT Conventional PAL	Dependable conventional (molded) progressive for an affordable, reliable everyday multifocal. MFH 14 / 17 mm · Conventional design

Specialty progressives

DESIGN	WHAT IT'S FOR	FIT NOTE
Endless Office	Indoor near + intermediate; desk and screens	3 ranges: 1.3 m desk, 2 m workstation, 4 m room
Endless Sport	Wrap-ready outdoor: golf, bike, boat	Pairs with polarized / mirror; give wrap + panto + vertex
Endless Anti-Fatigue	Digital comfort; younger eyes, heavy screens	Single-vision base with a relaxing near boost
KEY TECHNOLOGY IOT Digital Ray-Path 2 (point-by-point ray tracing) + Steady Methodology. Spatial Vision optimizes ~99.5% of gaze directions; Visual Stability cuts swim; Eye Focus reduces peripheral blur.		

Understanding a progressive lens

A progressive addition lens (PAL) blends three zones — distance at the top, an intermediate corridor, and near at the bottom — with soft, unusable optics at the lower corners. These engraved and inked markings let you verify and re-mark any PAL.



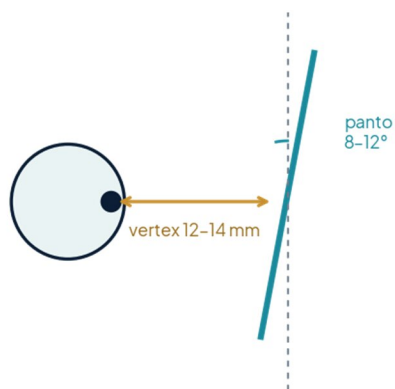
Anatomy of a progressive lens: zones, reference points and engravings.

1. Fitting Reference Point (FRP) — the fitting cross; sits on the pupil centre. Re-create with a layout chart if erased.
2. Prism Reference Point (PRP) — 4 mm below the FRP; the optical centre, used to check prism (prescribed or thinning).
3. Two micro-circles — engraved 17 mm each side of the PRP; verify axis and re-create the FRP.
4. Design identifier — engraved under the nasal micro-circle; unique to each PAL design.
5. Material identifier — to the right of the design mark; denotes the lens material.
6. ADD engraving — under the temporal micro-circle; ALWAYS read the add here, never off the lensometer.
7. Distance & near verification circles — check distance power above; the near circle no longer proves total near power because insets and progression lengths vary.

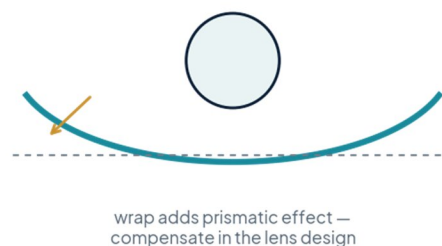
Fitting progressives

Good optics start with good position-of-wear. Capture monocular PDs and fitting heights on the frame the patient will wear, and record the at-wear geometry below — small errors here cause most non-adapts.

PANTOSCOPIC TILT & VERTEX



FACE-FORM WRAP



Pantoscopic tilt, vertex distance and face-form wrap all change the effective prescription.

PARAMETER	TYPICAL AT-WEAR	WHY IT MATTERS
Pantoscopic tilt	8–12°	Too little softens distance & near; too much induces error
Vertex distance	12–14 mm	Affects effective power, especially in higher Rx
Face-form / wrap	Frame dependent	Wrap induces prism — must be compensated in design
Monocular PD & heights	Per eye	Off-centre optics = narrow corridor, swim, off-axis blur

Fast fitting—issue remedies

PATIENT REPORTS	DO THIS
Narrow reading area	Verify height & PD, verify ADD, add panto tilt, reduce vertex
Peripheral vision blurs / moves	Reduce vertex distance, increase facial wrap
Lifts head/glasses to read (fit too low)	Raise frame on face, nose pads closer, reduce panto; refit if needed
Lowers head/glasses to read (fit too high)	Lower frame, widen nose pads, increase panto; refit if needed
Moves reading material to the side	Check lenses sit on iris, verify monocular PD, remake with correct PD
Distance slightly blurry	Increase panto tilt, decrease fitting height

Dispensing progressives

Adaptation is taught, not left to chance. Set patient expectations at the order decision point and then at collection and coach the patient through the three zones. A patient who knows what to expect becomes an advocate; one who doesn't becomes a return or non-adapt.

Coach the patient at order and at collection

- Look far, then lower your eyes — not your head — to bring closer things into focus.
- For reading, drop your gaze to the bottom of the lens and lift the book a little.
- Point your nose at what you want to see; move your head, not just your eyes.
- The lower corners will always look soft — that's normal, not a fault.

Advice for wearing

- Choose larger, full-frame styles — the near zone needs vertical room, and a full frame hides thicker edges.
- Expect about a week to adapt; walk carefully at first and take stairs cautiously while looking through the distance zone.
- Wear them full-time during adaptation rather than switching back and forth to and from old glasses.

SET THE FRAME EXPECTATION EARLY

Two identical designs in a larger vs. smaller frame give different clear areas — a few millimetres matters. Tell the patient at the dispense, not at the return.

Troubleshooting progressive lenses

When a patient can't adapt, work the problem systematically rather than guessing.

Four steps

1. Have a plan, make a comparison — on a paper write old pair vs. new pair side by side: Rx (the add on its own row), fitting parameters (OCs, heights, BVD, panto, wrap), design, progression length, prism thinning, material, treatments. (Ignore base curve.)
2. Analyze the data — what changed and by how much? Work Rx → measurements → design. Take note of or highlight the big shifts.
3. Explain — tell the patient what you're checking at each step. A concern left unexplained becomes a complaint.
4. Seek advice — your lab has seen hundreds of thousands of jobs. Send your analysis; we'll give you the solution.

Common issues, causes & remedies

ISSUE	LIKELY CAUSE	REMEDY
Tilting head up to read	Add too weak / PD wrong / corridor too long	Verify add & PD; match previous MFH; shorter corridor
Tilting head up for distance	Under-corrected hyperope / pupil not centred	Repeat refraction; verify pupil at DRP; remake if needed
Looking to the side to focus	Lenses mounted wrong / PD wrong	Verify pupil at design reference point; verify PD; remake
Head forward to read	Add too strong / seg too low / low panto	Repeat refraction; verify seg height; add panto
Swimming sensation	PD/seg wrong / low panto / low wrap / design too hard	Verify PD & seg; add panto & wrap; try a softer design
Reading area too small	PD wrong / seg too low	Verify PD & seg height; adjust frame; remake if needed

ESSENTIAL KIT & A FINAL TIP

Keep flipper lenses, a marker and a mirror at hand. Mark the NVP (Near Vision Position), have the patient hold the chart on the mirror at reading distance, stand behind and watch that they use the near zone. And don't over-read isocylinder maps (progressive design maps)— any single chart is meaningless in isolation.

The myth of increasing add power

Why it's not a solution for progressive users with reading or adaptation issues. **By Russell Hunte**

When patients struggle to adapt, a common suggestion — sometimes from other spectacle wearers — is to "just bump up the add." It feels intuitive, and it is wrong. Add power is the extra magnification at the bottom of the lens for near vision; it is prescribed from clinical need, not from frustration.

Why more add makes things worse

- Narrower intermediate: a stronger add shrinks the arm's-length zone patients need for screens and everyday tasks.
- Eyestrain & headaches: over-magnifying near forces the eyes to an unnatural level and adds fatigue.
- Harder adaptation: raising the add before the patient is ready fights the smooth zone-to-zone transition PALs rely on.
- Long-term dependence: pushing add too early hastens reliance on stronger lenses as presbyopia naturally progresses.

What to do instead

1. Educate on proper use — most struggles are simply not knowing which part of the lens to use for which task.
2. Allow a gradual adjustment period — short spells building up, worn full-time, not switched off.
3. Reassess fit & design — the issue is often height, frame or design, not power. Re-check position-of-wear.
4. Use a task-specific lens — for heavy computer or close work, an occupational (Endless Office) lens beats over-adding a general PAL.

THE TAKEAWAY

Increasing add power as a reaction to frustration is a myth that trades one problem for several. Trust the process: adaptation, education and an individualized fit deliver clear, comfortable vision at every distance without compromising the patient's eyes.

CHOOSING THE RIGHT MULTIFOCAL

Progressives vs bifocals

Bifocals put two fixed powers — distance above, near below — in one lens, separated by a visible line. They are simple and inexpensive, but the hard line creates "image jump," offers no true intermediate (arm's-length) vision, and is cosmetically dated. For most presbyopes the progressive has replaced them.

A progressive blends distance, intermediate and near in a single smooth surface with no line and no jump — the wearer simply glides their gaze down to move nearer. It looks like an ordinary single-vision lens and, in a digital free-form design like Endless Steady, gives wide, stable fields at every distance.

	CONVENTIONAL BIFOCAL (ADEPT)	ENDLESS PROGRESSIVE (BEST)
Intermediate / screens	Poor — no dedicated zone	Dedicated smooth corridor
Cosmetics	Visible line	No line; looks single-vision
Image jump	Yes, at the segment	None
Adaptation	Immediate but limited	~1 week; far more capable
Best use	Budget, simple near tasks	Everyday all-distance vision

Where a bifocal still earns its place

- True progressive non-tolerances who still need wide, stable near — consider the Wide-Seg Blended Bifocal.
- Very specific single-distance near tasks on a tight budget.
- Always frame it as a considered choice, never a downgrade — and never call a progressive a "no-line bifocal."

Single vision

BEST Endless SV

Fully personalized free-form single vision — impeccable clarity for high plus/minus prescriptions, wrap frames and heavy digital use.

Point-by-point optimized

ADEPT Conventional SV

Dependable everyday single vision across all materials and treatments — the value choice.

All materials & tints

Match the material to the Rx (see Materials) and recommend Super AR+ on higher-index lenses.

SEGMENTED MULTIFOCALS

Bifocals & trifocals

BEST Endless Bifocal

Digitally surfaced free-form bifocal — smoother, wider, more usable near than a legacy molded flat-top.

Free-form

SPECIALTY Wide-Seg Blended Bifocal (60 mm)

Digitally surfaced blended bifocal with a ~60 mm near zone and a soft, semi-invisible line. Wide reading, modern look, lighter than an Executive.

Progressive non-tolerances; readers, drivers, trades

ADEPT Conventional BF / TF

Trusted flat-top bifocal & trifocal formats — practical, familiar, cost-effective.

FT-28 / FT-35 / round seg

Wide-Seg Blended Bifocal — quick fit notes

- Frame B-size ≥ 32 mm; at-wear panto 8–12°, vertex 12–14 mm; monocular PDs and seg heights at lower limbus.
- Readers seg –1 to –2 mm; drivers/tall posture seg +1 mm; near inset ~2.5–3.0 mm per eye.
- Use caution with heavy intermediate needs (offer Endless Office), anisometropia > 2.00 D, or adds $\geq +3.00$.
- Typical adaptation 1–3 days; premium AR recommended by default.

CHOOSING THE LENS MATERIAL

Materials & thickness

INDEX / MATERIAL	ABBE	DENSITY	BEST FOR
1.50 CR-39	58	Low	Economy, low Rx, easiest to tint
1.53 Trivex	~45	Lowest	Rimless / drill, impact, light weight
1.56 Mid-Index	42	Low	Value; a little thinner than 1.50
Polycarbonate ~1.59	~30	Low	Safety, kids, sports, rimless
1.60 MR-8	42	Medium	Moderate Rx, thinner profile
1.67 MR-7	32	Medium	Higher Rx, notably thinner
1.74 MR-174	33	Medium	Highest Rx, thinnest available

Quick thickness guidance

- The higher the Rx power, the higher the index needed to keep lenses thin and light.
- Strong minus (myopia): edge thickness is the issue — step up to 1.67 / 1.74 and choose a smaller frame.
- Strong plus (hyperopia): centre thickness is the issue — high index + aspheric flattens the lens.
- Lower Abbe (higher index) means more chromatic aberration — always pair high index with Super AR+.
- Impact-first jobs (kids, safety, rimless): polycarbonate or Trivex regardless of Rx.

LIGHT-REACTIVE & SUN

Photochromic, polarized & Transitions

Photochromic families — they are not interchangeable

PRODUCT	BEHAVIOUR	POSITION IT FOR
Transitions GEN S / GEN 8	Fast, everyday clear-to-dark	General daily photochromic
Transitions XTRActive	Darker; activates behind the windscreen	Drivers, brighter climates
XTRActive Polarized	Photochromic + polarization when dark	Outdoor glare + variable light
Neochromes	Lower-cost XTRActive-style option	Budget photochromic
ZenVue Darkun	House photochromic	House-brand alternative

Polarized & Drivewear

- Polarized kills reflected glare off water, road and snow — the go-to for boaters, drivers and bright outdoors.
- Always add Back AR to polarized/mirrored lenses to stop reflections bouncing off the back surface into the eye.
- Drivewear combines polarization with photochromic behaviour tuned for driving light.
- Mirror coatings suit polarized or tinted lenses only — not clear or standard photochromic; for style mirrors use XTRActive.

Coatings & anti-reflective

Coatings are where good jobs are won or lost. Learn the house AR family below; the Crizal range is below ours.

COATING	WARRANTY	WHAT IT DOES
Scratch (SRC / HC)	—	UV-cured hard shield; makes lenses, especially high-index, scratch-resistant
Standard AR	1 year	Removes green/yellow glare; hydrophobic & anti-static
Blue Defense AR+	2 years	Guards against blue-violet light; non-glare, hydrophobic, anti-static
Super AR+	2 years	Premium, most transparent, durable; minimal reflective hue
Back AR	1 year	Cuts backside reflections on polarized / front-mirrored lenses
Mirror coating	2 years	Style statement for polarized/tinted; not for clear/standard photochromic

AR REMOVAL REALITY-CHECK

AR strip only works when the AR is breaking down — it dissolves the failed AR, not the hard coat beneath. Not recommended for badly scratched lenses.

PREMIUM BRANDED AR

Crizal coatings

Crizal is Essilor's premium anti-reflective family. Sell it on clarity, cleanability and warranty, and match the grade to the patient.

CRIZAL GRADE	POSITION IT FOR
Crizal Easy (Pro)	Entry-level branded AR; scratch resistance + UV
Crizal Alizé UV	Enhanced non-glare, smudge-resistant, easy to clean
Crizal Rock UV	Durable non-glare with superior scratch resistance
Crizal Avancé UV	Advanced everyday AR
Crizal Sapphire HR / 360	Most transparent AR; reflection reduction from all angles
Crizal Prevencia	Selective blue-violet + UV protection
Crizal SunShield / Style Mirrors	Back/mirror AR for sun lenses
Crizal Kids UV	Durable, non-glare, scratch-resistant for children

NAMING DISCIPLINE

Crizal is the brand line; Super AR+ / Blue Defense AR+ / Standard AR are the house line. Quote one system per lens — don't blend house and Crizal names in the same recommendation.

ENHANCED SINGLE VISION

Eyezen

Eyezen is Essilor's enhanced single-vision family — sharper than ordinary SV and built for a screen-heavy world. Position it as the everyday upgrade for anyone who isn't yet presbyopic but lives on devices.

- Eyezen enhanced SV: sharper vision than ordinary single vision, available to everyone across the Rx range.
- Eyezen+ : adds a small accommodative boost at the bottom of the lens to relax the eyes during prolonged near/screen work.
- Pair with Blue Defense AR+ (or Crizal Prevencia) for screen-heavy patients.
- Sits between Conventional SV (Adept) and Endless SV (Best) — the enhanced middle choice.

WHO TO OFFER IT TO

Students, office staff and anyone reporting end-of-day eye tiredness who does not yet need a progressive. It is an easy, high-value upgrade from a plain single-vision lens.

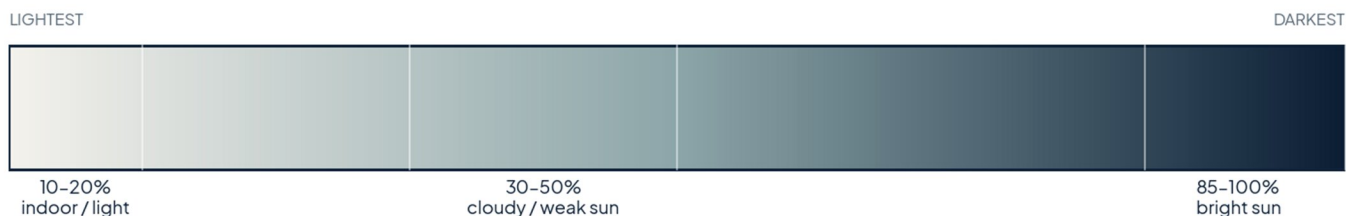
COLOUR, INTENSITY & MATERIAL

Tints

Popular tint colours

COLOUR	GOOD FOR
Brown	All-round glare & UV protection; enhances contrast
Gray	Neutral colour perception; true-to-life glare reduction
Green	Light sensitivity; reduces eye strain, good contrast
Yellow	Depth & contrast in low light; overcast / indoor sport
Pink / fuchsia	Reduces glare, subtle style; can help night driving with AR

Tint intensity



Intensity runs 10% (lightest) to 100% (darkest); match it to the patient's light exposure.

How material affects tinting

- CR-39 tints easily and takes the widest colour range — the most forgiving material.
- Polycarbonate resists dye; it's tinted via its scratch coat, so colour range and depth are limited.
- Trivex tints but not deeply; harsh cleaners (alcohol) fade it — use a tintable hard coat.
- High-index tints, but slowly — a 10-minute CR-39 tint can take 3-4 hours on high index.
- UV coat before colour; tint ~10% darker before AR; replace both lenses together for a colour match.

Chemistrie clip system

Chemistrie (Eyenavision) adds custom, magnetically attached clip layers over the patient's own Rx — one frame, many uses, applied only when needed. Ordered through a Chemistrie-certified lab.

CLIP	USE IT FOR
Sun (solid / gradient / mirror)	On-demand polarized sun over clear Rx
Night Driving	Cuts oncoming glare for night drivers
Blue	Reduces ~380–450 nm blue for screen-heavy days
Plus (readers)	Adds near power over distance Rx
Color	Colour-correction / style

Therapeutic clips — light sensitivity & migraine

- Chemistrie Avulux: spectral nanofilter that filters up to ~97% of blue/amber/red light linked to migraine triggers while passing >70% of soothing green — colour-accurate, on-demand.
- Chemistrie Calm (FL-41): therapeutic FL-41 tint in Light and Medium for photophobia and fluorescent/LED sensitivity.
- Pathway: treat ocular surface & migraine first; trial Avulux indoors for spectral relief; use FL-41 if the patient prefers it; add a polarized sun clip for bright outdoors.

DISPENSER WORKFLOW

Fit the patient, send frame specs + Rx to a Chemistrie-certified lab, receive the custom clip. Great for patients who need relief only some of the time and want to keep clear Rx eyewear for everyday.

RECENTLY ASKED

Frequently asked questions

Why can't I find a price for a lens I bought before?

One of three reasons: the lens was discontinued; it was missed in error; or it has been replaced by a newer lens. Ask us and we'll point you to the current equivalent.

The patient needs a reorder but has no spare pair — what now?

If we still have the original shape we can remake it if the frame is sound. Otherwise send a photo with the order so we can verify the shape matches.

Why would a patient need an office progressive if they already have progressives?

They serve different jobs. A general progressive covers all distances; Endless Office is optimized for intermediate and near — computer and desk work — giving wider task zones, a more natural head/neck posture, and less strain during long screen sessions. It complements, not replaces, their everyday pair.

How do I choose the right lens style for a patient?

There's no single right answer — it depends on what you ask and what you learn about their needs, tasks and priorities. Ask about their day, their screens, their hobbies and their previous lenses, then match the tier and design.

Quick recommendation guide

PATIENT / SCENARIO	RECOMMEND
"My eyes are tired from screens all day"	Endless Steady or Endless Anti-Fatigue + Blue Defense AR+
First-time progressive, price sensitive	Classic PAL or Essential Steady
Not presbyopic yet, heavy screen user	Eyezen + Blue Defense AR+
Strong Rx, wants thin & light	1.67 or 1.74 + Super AR+
Child / safety / active	Polycarbonate or Trivex
Rimless or drill-mount frame	Trivex
Outdoor glare / driving	Polarized + Back AR
Night-driving trouble	Chemistrie Night Driving clip
Migraine / light sensitivity	Chemistrie Avulux or Calm (FL-41)
Office worker, desk + monitors	Endless Office (2 m)
Golfer / boater	Endless Sport + polarized or mirror
Progressive non-tolerant, needs wide near	Wide-Seg Blended Bifocal
Wants the clearest, reflection-free look	Crizal Sapphire 360 / Super AR+

GET THIS RIGHT

Warranties & policies every dispenser must know

TOPIC	WHAT TO TELL THE PATIENT / DO
Progressive non-adapt	We replace with SV or lined bifocal lenses free once per job within 60 days; edging/tint/coating charges still apply. Not combinable with Doctor's Change.
Doctor's change	Prescriber error: one-time 25% courtesy on replacement lenses within 60 days, same frame & style; services still apply.
Coating warranty	AR & specialty coatings: 1-year warranty vs. manufacturer defects (peel/bubble/failure); return within 12 months for inspection.
Processing time	Allow up to 15 working days for AR-coated or digital lenses; overseas-sourced may take longer.
Uncut breakage	If you break an uncut during edging, one replacement per job at 30% off lens cost.
Customer-supplied frames	Used frames at customer risk; get the waiver signed before working on frames not bought from us.
Lens storage	Uncut lenses awaiting frames stored max 30 days, then returned; warranty runs from original delivery date.

GOLDEN RULE OF ACCURACY

The customer is responsible for the accuracy of Rx, frame details and measurements — but you are the last line of defence. Verify before you order; a two-minute check beats a two-week remake.

Glossary & how to order

Terms a new dispenser should own

TERM	MEANING
PAL	Progressive Addition Lens
FRP / PRP	Fitting Reference Point (cross) / Prism Reference Point
MFH / seg height	Minimum Fitting Height / segment height — vertical fitting position
ADD	Near addition power at the bottom of a multifocal
POW / BVD	Position Of Wear / Back Vertex Distance (vertex)
Panto	Pantoscopic tilt — forward tilt of the lens plane
Corridor	The intermediate zone length between distance and near in a PAL
Abbe	Measure of chromatic aberration — higher is better (less colour fringing)

How to order & who to ask

Order and track through the LabLink portal, or contact the lab directly. When you're stumped on a fit or a non-adapt, your first call is us — send your two-column analysis and we'll give you the solution.

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